

1. Administrative & Study Identification

Full Study Title: Open Label, Multi-center, Phase IV Study of Ruxolitinib or Ruxolitinib and Panobinostat Combination, for Patients Who Have Completed Prior Global Novartis or Incyte Sponsored Studies **Short Title/Acronym:** CINC424A2X01B Rollover Protocol **Protocol Number:** CINC424A2X01B (EudraCT: 2014-003527-22) **NCT Number:** NCT02386800 **PostingID:** 931669158353417708 **Sponsor/Company Name:** Novartis Pharmaceuticals (Novartis) **Investigational Product:** Ruxolitinib (INC424 / Jakavi) monotherapy or panobinostat (drug preferred name: panobinostat; trade name: Farydak; ATC Code: L01XH03) in combination with ruxolitinib **Phase of Development:** Phase IV **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the long-term safety data (incidence and severity of AEs and SAEs) of patients taking ruxolitinib or a combination of ruxolitinib and panobinostat. **Secondary Objectives:** 1. Proportion of participants with clinical benefit as assessed by the investigator at scheduled visits. 2. Incidence and severity of AEs and SAEs by treatment group.

2.2 Background & Rationale This roll-over protocol enables patients from multiple parent protocols—who are still receiving clinical benefit from their targeted therapies—to continue their treatment in a single, consolidated study. Covering multiple indications (e.g., Primary Myelofibrosis, Polycythemia Vera, Graft Versus Host Disease), this study aims to characterize the long-term safety profile of the treatments after the parent studies have concluded, provided the investigator determines the patient should continue therapy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment, Duration & Status Trial Status: ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Target \$N\$:** 279 patients (Targeted Enrollment) **Number of Sites:** Multi-center (Global representation including USA, Europe, Canada, Japan, and RoW) **Estimated Study Start:** 05-Mar-2015 **Estimated Primary Completion:** 16-Sep-2027

4. Subject Selection (Eligibility Criteria)

General Criteria Requirement: PHASE_REQUIREMENT (Trial must be in PHASE4)

4.1 Inclusion Criteria 1. Patient is currently enrolled in a Novartis GDD or GMA-sponsored or Incyte-sponsored parent clinical study, is receiving either ruxolitinib or a combination of ruxolitinib and panobinostat, and has fulfilled all parent protocol requirements. 2. Patient is currently benefiting from the treatment with ruxolitinib monotherapy or combination of ruxolitinib and panobinostat, as determined by the investigator. 3. Patient has demonstrated compliance, as assessed by the investigator, with the parent study protocol requirements. 4. Patient currently has no evidence of progressive disease, as determined by the investigator, following previous treatment with ruxolitinib or combination of ruxolitinib and panobinostat.

4.2 Exclusion Criteria 1. Patient has been permanently discontinued from study treatment in the parent study due to any reason. 2. Patient's indication is currently approved and reimbursed in the corresponding local country for ruxolitinib monotherapy or the panobinostat combination. 3. Pregnant or nursing (lactating) women. 4. Female patients of childbearing potential (e.g., are menstruating) who do not agree to abstinence or, if sexually active, do not agree to the use of highly effective contraception throughout the study and for up to 30 days after stopping study treatment.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Participants will continue to receive ruxolitinib monotherapy or ruxolitinib in combination with panobinostat at the exact same dose and schedule they were taking in the parent study. **Route of Administration:** Oral (ruxolitinib tablets/pediatric oral solution; panobinostat capsules). **Reference Information:** Panobinostat EMA Product Information is available at: https://www.ema.europa.eu/en/documents/product-information/farydak-epar-product-information_en.pdf **Duration of Treatment:** Long-term safety follow-up spanning up to approximately 12 years (until disease progression, loss of clinical benefit, or estimated completion in 2027).

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care as outlined in the respective parent protocols. **Prohibited:** Use of new investigational agents outside the designated rollover regimen or therapies contraindicated with JAK1/2 or HDAC inhibitors.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -14 to 0	Informed Consent, verification of parent study completion, confirmation of absence of progressive disease.
Baseline	Day 0	Rollover enrollment, continuation of parent study dosing.
Interim	Ongoing	Safety evaluations (AEs/SAEs), ongoing evaluation of clinical benefit.
End of Study	Upon Exit	Final safety follow-up spanning 30 days after stopping study treatment.

(Note: Efficacy parameters are explicitly not measured per protocol; the focus remains entirely on long-term safety and investigator-assessed benefit.)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence and severity of AEs and SAEs. **Measurement Method:** The incidence of treatment-emergent AEs and SAEs (new or worsening from baseline) will be summarized by system organ class and/or preferred term, severity (based on CTCAE grades).

7.2 Secondary & Exploratory Endpoints 1. Proportion of participants with clinical benefit as assessed by the investigator at scheduled visits. 2. Incidence and severity of AEs and SAEs by treatment group.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of treatment-emergent AEs across all participating global sites.

Serious Adverse Events (SAEs): Monitored continuously with severity tracked according to standard CTCAE diagnostic guidelines. **Data Monitoring Committee (DMC):** Maintained according to standard Novartis Phase IV pharmacovigilance and safety protocols.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all rollover patients receiving at least one dose under the new protocol). **Sample Size**

Justification: The sample size (\$N=279\$) was not based on statistical power for efficacy, but rather driven by the number of eligible patients completing the parent protocols who continued to derive clinical benefit. **Handling of Missing Data:** Summarized descriptively for safety data; missing efficacy data is not imputed as efficacy parameters are excluded from measurement in this rollover design.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine multi-center sponsor monitoring to ensure adherence to safety reporting, capturing adverse events, and assessing ongoing clinical benefit. **Audit Trail:** eCRF logging for protocol compliance, tracking parent-to-rollover transitions, and securing long-term safety datasets.

11. Study Identifiers & Governance

Primary ID: CINC424A2X01B **Registry Number:** NCT02386800 **PostingID:** 931669158353417708 **Sponsor/Lead Organization:** Novartis **Study Title:** CINC424A2X01B Rollover Protocol **Official Regulatory Title:** Open Label, Multi-center, Phase IV Study of Ruxolitinib or Ruxolitinib and Panobinostat Combination, for Patients Who Have Completed Prior Global Novartis or Incyte Sponsored Studies **Source Level:** 5

12. Clinical Framework (Disease Specific)

Preferred UMLS Name: Hematologic Diseases **Primary Conditions / Disease Areas:** Primary Myelofibrosis (Preferred Name: Primary Myelofibrosis), Polycythemia Vera, Graft Versus Host Disease, Acute Myeloid Leukemia, Thalassemia **Semantic Type:** Neoplastic Process **Disease Definition (Primary Myelofibrosis):** A chronic myeloproliferative neoplasm characterized by bone marrow fibrosis, proliferation of atypical megakaryocytes and granulocytes in the bone marrow, anemia, splenomegaly, and extramedullary hematopoiesis. (WHO, 2001) **MeSH Code:** D055728 **SNOMED ID:** 128843003 **Diagnosis Threshold:** Previous definitive diagnosis mapped in parent study and current lack of progressive disease. **Medical Methodology:** Long-term maintenance of targeted JAK1/2 inhibition ($\pm$ HDAC inhibition). **Optimization Target:** Sustained clinical benefit and long-term safety.

13. Study Procedures & Methodology

Management Pathway: Continued administration of established, well-tolerated targeted therapies from parent trials. **Education Protocol (HCP):** Guidance on managing and reporting long-term safety signals for ruxolitinib and panobinostat. **Education Protocol (Patient):** Counseling on long-term safety follow-ups, reporting AEs, and strict contraceptive guidelines. **Quality Audit Mechanism:** Global sponsor-led pharmacovigilance tracking across diverse indications.

14. Observation & Follow-Up Schedule

Total Duration: Estimated 12 years (Target Completion: September 2027) **Onsite Visit Cadence:** Maintained at standard-of-care or parent-study intervals to allow rigorous ongoing AE/SAE assessment. **Remote Monitoring Frequency:** In accordance with standard continuous pharmacovigilance protocols. **Checklist Review Interval:** Continuous clinical benefit evaluations by the investigator at routine dosing/dispensing visits.

15. Sign-off & Approval

Role	Name	Signature	Date	---	---	---	---
Principal Investigator	[Insert Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Insert Name]	_____	[DD-MMM-YYYY]				
Lead Statistician	[Insert Name]	_____	[DD-MMM-YYYY]				